

Case 1: Bipolar I Disorder

ID: A.R., **Age:** 41 years, 2 months, 3 days, **DOB:** 05/12/1985,
Effective: 07/15/2026, **Diagnoses:** Bipolar I Disorder, Current
Episode Manic, Severe

CC: Monthly medication management follow-up.

Subjective:

"I'm feeling fantastic, I haven't slept much but I have an unbelievable amount of energy and I'm starting a new tech company."

The patient reports highly increased energy levels, decreased need for sleep (sleeping roughly 2-3 hours per night for the past week), and racing thoughts. They report spending excessively on new business ventures. Denies any feelings of depression or lethargy at this time.

Objective

Lithium serum level: 0.4 mEq/L (Subtherapeutic)

Comprehensive Metabolic Panel (CMP): Within normal limits

TSH: 1.8 mIU/L (Within normal limits)

Mental Status Evaluation:

- **General Appearance:** ☒ Well-groomed ☐ Disheveled
- **Eye Contact:** ☒ Intense ☐ Normal ☐ Avoidant
- **Stature:** ☒ Average weight ☐ Underweight ☐ Overweight

- **Accessibility:** ☒ Readily engaged ☐ Withdrawn
- **Style:** ☐ Flamboyant ☐ Casual ☐ Unremarkable
- **Psychomotor activity:** ☒ Hyperactive/Agitated ☐ Normal
- **Sensorium:** ☐ Clear and oriented x4 ☐ Clouded
- **Speech:** ☐ Rapid rate, ☒ Pressured, ☐ Regular rhythm, ☐ Loud amplitude, ☐ Clear articulation, ☒ Hypervocal style
- **Mood:** "I'm feeling fantastic, I haven't slept much but I have an unbelievable amount of energy and I'm starting a new tech company."
- **Rapport:** ☐ Easily established ☐ Difficult to establish
- **Affect:** ☒ Euphoric ☐ Euthymic ☐ Blunted
- **Cognition/Memory:** ☐ Grossly intact ☐ Impaired
- **Suicidal ideation:** ☐ No ☐ Yes
- **Homicidal ideation:** ☐ No ☐ Yes
- **Insight:** ☐ Good ☐ Fair ☒ Poor
- **Judgment:** Impaired ☒ Yes ☐ No
- **Abstract thinking:** ☐ Intact ☐ Concrete
- **Comments:** Patient is exhibiting classic signs of acute mania, likely exacerbated by a subtherapeutic lithium level.

Assessment: Bipolar I Disorder, Current Episode Manic.

Plan

Target Symptoms:

- Decreased need for sleep
- Hyperactivity and racing thoughts
- Grandiosity

Psychotropic Medications:

- Lithium Carbonate ER 600 mg PO BID
- Quetiapine 100 mg PO QHS

Medication Changes:

- Increase Lithium Carbonate ER to 900 mg PO QPM and 600 mg PO QAM to achieve therapeutic blood levels.
- Increase Quetiapine to 200 mg PO QHS temporarily for sleep induction and acute manic symptom control.

Other Interventions:

- Educated patient on strict sleep hygiene protocols.
- Recommended delaying any major financial decisions until mood stabilizes.
- Return to clinic in 1 week for an acute follow-up and repeat Lithium level draw.

Case 2: Schizophrenia

ID: M.K., **Age:** 33 years, 7 months, 25 days, **DOB:** 11/20/1992,

Effective: 07/15/2026, **Diagnoses:** Schizophrenia

CC: Monthly medication management follow-up.

Subjective:

"The neighbors are beaming radio signals into my television again, trying to read my thoughts."

The patient reports ongoing, distressing auditory hallucinations (hearing two voices commenting on his daily actions) and paranoid delusions regarding his apartment neighbors. He notes that medication compliance has been intermittent because the pills make him feel "too slow."

Objective

Fasting Lipid Panel: Within normal limits

Fasting Glucose: 92 mg/dL

Abnormal Involuntary Movement Scale (AIMS): Score 0 (No tardive dyskinesia noted)

Mental Status Evaluation:

- **General Appearance:** ☐ Well-groomed ☒ Disheveled
- **Eye Contact:** ☐ Normal ☐ Intense ☒ Poor/Avoidant
- **Stature:** ☒ Average weight ☐ Underweight ☐ Overweight

- **Accessibility:** ☐ Readily engaged ☒ Guarded
- **Style:** ☐ Unremarkable ☒ Eccentric ☐ Casual
- **Psychomotor activity:** ☐ Normal ☒ Slowed/Retarded
- **Sensorium:** ☒ Clear and oriented x4 ☐ Clouded
- **Speech:** ☒ Slow rate, ☐ Pressured, ☒ Regular rhythm, ☒ Soft amplitude, ☒ Clear articulation, ☒ Poverty of speech style
- **Mood:** "The neighbors are beaming radio signals into my television again, trying to read my thoughts."
- **Rapport:** ☐ Easily established ☒ Difficult to establish
- **Affect:** ☐ Euthymic ☒ Flat/Blunted ☐ Labile
- **Cognition/Memory:** ☒ Grossly intact ☐ Impaired
- **Suicidal ideation:** ☒ No ☐ Yes
- **Homicidal ideation:** ☒ No ☐ Yes
- **Insight:** ☐ Good ☐ Fair ☒ Poor
- **Judgment:** Impaired ☒ Yes ☐ No
- **Abstract thinking:** ☐ Intact ☒ Concrete
- **Comments:** Patient is experiencing active psychosis and prominent negative symptoms. He appears internally preoccupied during periods of silence.

Assessment: Schizophrenia.

Plan

Target Symptoms:

- Auditory hallucinations
- Paranoid delusions
- Medication side effects (sedation)

Psychotropic Medications:

- Risperidone 4 mg PO QHS

Medication Changes:

- Decrease Risperidone to 3 mg PO QHS to help mitigate morning sedation and encourage better adherence.
- Will consider cross-tapering to a weight-neutral, less sedating atypical antipsychotic if compliance remains an issue next visit.

Other Interventions:

- Referred to community outreach/case management program for assistance with daily living skills.
- Discussed the importance of daily medication adherence to quiet the auditory hallucinations.
- Return to clinic in 4 weeks.